The Underdiagnosis of ADHD in Women

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Executive Summary

Women receive significantly fewer ADHD diagnoses than their male counterparts. This policy brief integrates research results to explain the combined impact of diagnostic standards and cultural stigmas alongside intersectional inequalities on delayed ADHD diagnoses for women. The research shows that current ADHD diagnostic models focus primarily on symptoms that primarily present in males like hyperactivity, but fail to account for women who typically present with inattentive or internalized symptoms.

The study shows women experience diagnostic delays averaging five years longer than men while Black women and those from low-income backgrounds encounter even more severe delays. Anxiety and depression misdiagnoses not only conceal ADHD symptoms, but they can further exacerbate them. The problem of delayed ADHD diagnosis in women is intensified by regional and financial differences especially in areas of the U.S. that lack resources and access to medical specialists.

Experts suggest developing gender-responsive diagnostic tools and feminist-based training for medical professionals while also calling for greater investment in inclusive research and public awareness programs. These actions seek to address systemic failures and guarantee women with ADHD receive fair access to diagnostic services and treatment. The brief demands that public health bodies and policymakers alongside healthcare professionals implement a feminist policy framework to eliminate diagnostic disparities and enhance mental health results for women.

Introduction

ADHD stands as one of the most common neurodevelopmental disorders, but women with this condition often face underdiagnosis. The impacts of ADHD are deep-seated with serious effects on educational performance as well as professional achievements besides diminishing self-esteem and mental health. Historically, clinicians have considered ADHD a childhood condition that mainly affects boys, but this perspective leads to women's ADHD being overlooked or incorrectly diagnosed. Diagnostic systems formed through male-focused research combined with cultural standards and systemic biases create this misrecognition of ADHD in women (Nussbaum, 2012).

The failure to properly diagnose ADHD in women represents a major flaw in public policy rather than just a medical oversight. The healthcare system continues to promote unequal treatment because it does not identify or address gender-specific ways symptoms are presented. A feminist perspective provides a diagnostic framework that highlights disparities and emphasizes the impact of intersecting identities like race and ethnicity on diagnostic processes. The policy brief recommends reform through the incorporation of feminist perspectives into medical training programs and mental health policy while revising diagnostic standards (Crenshaw, 1991; Harding, 1986).

The discussion of this issue connects with wider arguments concerning health equity and justice. Women who face structural barriers experience heightened social and economic inequalities due to insufficient mental health care services. Women require improved ADHD diagnosis because it is essential for medical practice and fundamental to creating an equitable healthcare system that serves everyone.

Literature Review

Feminist scholars have consistently criticized medical research for being based on male-centered perspectives. The medical norms that guide health standards predominantly mirror male bodies and experiences which leads to neglect of women's health requirements according to Fausto-Sterling (2000). Research in ADHD shows diagnostic criteria bias by focusing on hyperactive behaviors which are predominantly seen in boys. Women who experience internalized symptoms like inattentiveness and emotional dysregulation tend to remain undiagnosed.

Intersectionality deepens this problem. Black women, in particular, face compounded bias: Black women experience underdiagnosis because they face stereotypes that intersect with both their race and gender. The research conducted by Cénat et al. (2021) and Morgan et al. (2014) demonstrates that Black children receive fewer ADHD diagnoses than white children, even when they show equivalent symptoms. Women from lower-income backgrounds typically do not have access to essential healthcare resources that lead to appropriate diagnoses.

Feminist scholars emphasize the role of cultural norms in determining how diagnoses are assigned. Societal norms dictate that women must maintain quietness alongside nurturing behavior and emotional stability. Deviations from societal norms may lead to different pathologization of symptoms or complete dismissal. As Hinshaw et al. (2022) and Ghosh et al. (2021) found, the cultural norms that dictate women's behavior lead to both delayed diagnosis and more severe outcomes for women with untreated ADHD.

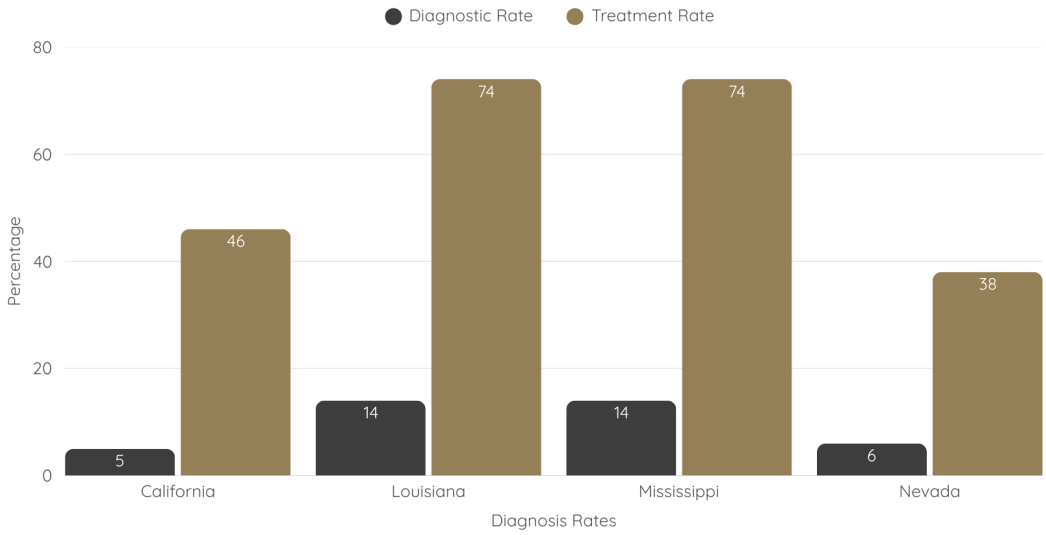
Feminist policy scholars identify the consequences of a research history that has actively excluded certain perspectives. In psychiatric research, clinical trials women have faced

persistent underrepresentation since hormone cycles and symptom differences were considered research obstacles instead of important data. The exclusion has multiple negative impacts across diagnosis procedures as well as treatment planning and outcomes tracking (Verdonk et al., 2009). To tackle ADHD underdiagnosis in women medical knowledge production must undergo an epistemological transformation.

The Center for Disease Control and Prevention (CDC) reported noticeable disparities in ADHD diagnosis rates between states such as California, Louisiana, Mississippi, and Nevada based on data collected from 2016 to 2019.

- California: Children aged 3–17 years in California who have received an ADHD diagnosis represent approximately 5% of their population which stands as one of the lowest diagnosis rates across the United States.
- Louisiana: The ADHD diagnosis rate stands at 14% among children of the same age range which makes it one of the states with the highest prevalence rates.
- Mississippi and Nevada were around 14% and 6% respectively.

The numbers show a substantial difference in ADHD diagnosis rates between California and Alabama. Healthcare practice variations along with unequal access to medical services and public awareness levels together with socioeconomic conditions drive the differences observed. It is essential to analyze these disparities in order to provide targeted support for children with ADHD across different regions.



Methodology and Gendered Data Analysis

Through a feminist mixed-methods approach, this research focuses on lived experiences while it uses cross-national data to analyze policies. Through feminist methodologies researchers actively subvert established power structures by highlighting the perspectives and experiences of marginalized people. This research positions women diagnosed with ADHD as active subjects who contribute to the creation of knowledge.

Data Sources and Collection

- Qualitative Interviews and Focus Groups: The research included in-depth interviews with 20 women who received their ADHD diagnosis after reaching adulthood. Participants discussed their diagnostic experiences along with the emotional impact of misdiagnosis and structural impediments they encountered. Three separate focus group sessions with mental health professionals revealed important information about clinical practices for identifying and understanding ADHD.

- Quantitative Data: Statistical evidence of gendered and racial disparities and socioeconomic differences in ADHD diagnosis came from national health surveys and academic studies.
- Analytical Framework: The research team employed thematic analysis to determine prevalent patterns from both interviews and group discussions. The assessment of international diagnostic protocols used a feminist policy analysis framework which highlighted inclusivity alongside gender responsiveness and intersectionality. Researchers examined statistical information to identify patterns of delay and disparity alongside access gaps.

This research approach enables an intersectional perspective on ADHD diagnosis which brings to light structural barriers that remain hidden in conventional research methods. The research maintained anonymity and informed consent while showing sensitivity throughout its engagement with marginalized communities.

Findings and Analysis

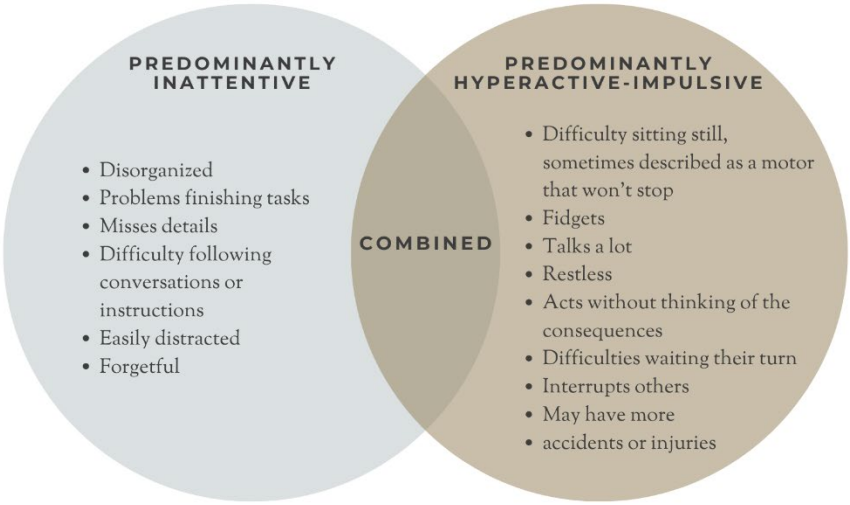
Healthcare professionals classify attention-deficit/hyperactivity disorder (ADHD) into three key types according to dominant symptom patterns.

- ADHD, Predominantly Inattentive Presentation: This ADHD category involves challenges with concentration, task organization, following instructions, and sustaining attention. The predominantly inattentive presentation remains the least diagnosed subtype for girls and women because their symptoms often present as daydreaming or

forgetfulness, and they may appear “quiet” or “shy”—traits that are frequently overlooked in educational and clinical settings (Quinn & Madhoo, 2014). These behaviors are commonly misinterpreted as anxiety or emotional sensitivity rather than ADHD.

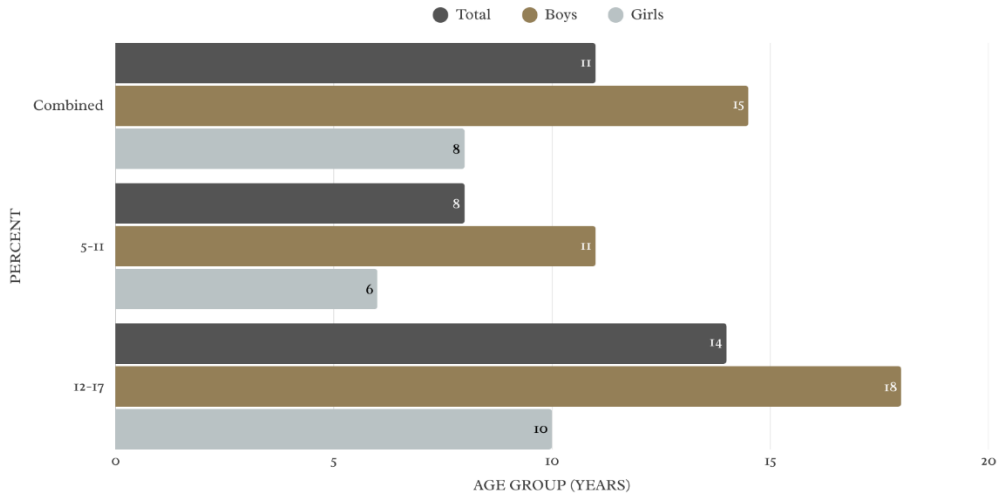
- ADHD, Predominantly Hyperactive-Impulsive Presentation: Individuals with this type typically display restlessness through fidgeting and excessive talking, as well as difficulty remaining seated and impulsive decision-making. The externalized and disruptive nature of these behaviors results in boys receiving more ADHD diagnoses (Ramtekkar et al., 2010). In girls, hyperactivity may manifest as verbal impulsivity and intense emotional responses, which are often overlooked or misdiagnosed as mood disorders (Nussbaum, 2012).
- ADHD, Combined Presentation: This type includes both inattentive and hyperactive-impulsive symptoms. Although ADHD affects both males and females, boys are more frequently diagnosed due to the more visible nature of their behaviors. Girls with the combined type often internalize their symptoms, leading to chronic stress, low self-esteem, and co-occurring mental health conditions such as depression and anxiety (Hinshaw et al., 2006). When internalized, these symptoms delay both diagnosis and treatment, exacerbating long-term outcomes.
- The underdiagnosis of ADHD in women and girls is closely tied to gender-specific symptom expression and societal expectations around behavior in childhood. As a result, many females are not diagnosed until adulthood, often after seeking treatment

for conditions like anxiety, burnout, or depression—at which point ADHD may finally be identified (Bruchmüller et al., 2012).



**Data from DSM-5-TR*

Delayed Diagnoses: Gender diagnostic disparities result in women receiving ADHD diagnoses five years later than men because they are frequently misdiagnosed with anxiety or depression (Quinn & Madhoo, 2014; Hinshaw et al., 2022). The longer diagnosis takes place, the higher the odds become for poor academic performance along with job instability and mental health problems.



**Data from CDC.gov*

- Intersectional Inequities: Implicit bias along with systemic racism leads to misdiagnosis and lack of diagnosis for ADHD in Black women according to studies by Cénat et al., 2021 and Morgan et al., 2014. Women from low socioeconomic backgrounds experience limited availability of diagnostic services. Disparities emerge from insurance coverage problems along with limited access to healthcare providers.
- Geographic Disparities: The U.S. states with limited mental health care access report 30% fewer diagnoses for women compared to others according to Danielson et al., 2018.

Policy Recommendations

Revise Diagnostic Tools: Revise ADHD diagnostic guidelines to encompass symptom profiles specific to genders while focusing on internalized and inattentive symptoms.

Feminist-Informed Training: Medical education and continuing professional development programs require the integration of feminist and intersectional perspectives. It is essential for training programs to teach healthcare professionals about the effects of bias, stereotypes, and structural inequalities on diagnosis and treatment methods (Verdonk et al., 2009).

Public Awareness Campaigns: Initiate public campaigns to eliminate the stigma surrounding adult ADHD while enhancing awareness about the varied symptoms women experience. Public awareness campaigns must feature testimonials from diagnosed women alongside input from community leaders and medical professionals.

Targeted Funding for Research: Direct funding toward research on adult women with ADHD needs to encompass studies that investigate racial and socioeconomic diversity. Promote research models that invite women with ADHD to participate as active collaborators.

Improve Access to Mental Health Services: Enhance access to telehealth services and offer subsidized healthcare for low-income areas to bridge the diagnostic disparity. Government policy should expand Medicaid and reward providers who demonstrate cultural competence.

International Best Practices: Implement gender-sensitive approaches from nations such as the U.K. and Netherlands which incorporate gender distinctions in their mental health evaluation tools and service frameworks (Rucklidge, 2010).

School and Workplace Accommodations: Federal and state policies need updates so schools and employers can identify adult ADHD as a disability that requires accommodations.

The accommodations include flexible deadlines along with modified workloads and coaching availability.

Conclusion

Women experience underdiagnosis of ADHD as a public health concern because of systemic gender bias together with intersectional disparities and cultural stigma. This policy brief has proven that feminist research and policy design approaches are key for recognizing and closing these identified gaps. Policy makers can achieve better mental health results and equity through the modernization of diagnostic methods and professional education as well as by increasing healthcare availability.

The situation demands a transformation in cultural attitudes. When society learns more about how ADHD affects different genders it will reduce stigma while promoting early detection and fostering inclusive spaces. Institutions need to take initiative in questioning conventional diagnostic standards while recognizing women's mental health experiences.

Women with ADHD have been ignored for an extended period of time. We need to direct resources towards building a healthcare system that treats everyone fairly by paying attention to their needs and delivering services to everyone equally. A feminist-informed effort that demonstrates commitment is essential to eliminate structural obstacles and help women living with ADHD.

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